

February 23, 2026

Robert F. Kennedy, Jr.
Secretary
U.S. Department of Health and Human Services
c/o Steven Posnack
Principal Deputy Assistant Secretary for Technology Policy, ASTP/ONC
7033A, 330 C Street S.W.,
Washington, DC 20201

ATTN: HHS Health Sector AI RFI, RIN 0955-AA13 – Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Secretary Kennedy and Principal Deputy Assistant Secretary for Technology Policy Posnack,

On behalf of Veradigm, we appreciate the opportunity to comment on HHS's Request for Information regarding actions the federal government can take to accelerate the adoption of artificial intelligence (AI) in clinical care. Veradigm has a proven history of delivering clinical and revenue cycle solutions that help independent practices improve outcomes and financial performance. Our AI-forward data and technology also help over 20,000 provider practices connect with health plans and biopharma to eliminate inefficiencies, close care gaps, and create a more affordable health system. These organizations play an essential role in American healthcare and often face distinct constraints, including limited staff, limited capital for digital transformation, and a disproportionately high administrative burden, which must be directly considered in any federal strategy to support scaled, responsible clinical AI adoption and use.

We believe AI has the potential to meaningfully improve patient care, clinician workload, and financial sustainability, but only when it is deployed with the right guardrails, supported by coherent regulatory expectations, and accessible to providers regardless of size or location. For this reason, Veradigm strongly supports a clear, harmonized federal framework for clinical AI that recognizes the unique operational realities of small, rural, and independent practices. Our comments reflect real-world implementation experience and align with our internal governance approach to AI at Veradigm. In general, our comments accentuate and expand on five (5) core themes:

1. We support adoption of a consistent, risk-based regulatory approach that differentiates high-risk clinical workflows. AI use cases in healthcare range from administrative automation to high-risk diagnostic support. Regulation should be proportionate to potential patient harm,

focusing the highest scrutiny on tools that directly influence diagnosis or treatment, while enabling rapid innovation in low-risk domains. We recommend the use of established risk analysis frameworks and clearly defined tiers and applicability thresholds.

2. Preserve clinical judgment and human-in-the-loop safeguards. AI should augment, not replace, clinical judgment. Core safeguards should include transparency standards related to model purpose, intended use, performance bounds, limitations, and training data characteristics. We also support design expectations that retain human decision-making in high-risk workflows and preserve clinician autonomy to override or dismiss AI recommendations when workflows are more automated or lower risk.

3. Support practical and scalable governance expectations for providers and developers. Governance expectations must be scalable to diverse care settings, from large health systems to small independent practices. We recommend flexible compliance pathways, such as simplified oversight structures for small practices; alignment of developer requirements with standard software development lifecycle practices; and harmonized federal guidance to reduce fragmentation across states and agencies.

4. Promote transparency without creating new liability exposure or barriers to innovation. Developers should be responsible for transparency and sound development practices; clinicians remain responsible for clinical use. HHS should clarify that transparency expectations do not reassign clinical liability and consider safe-harbor provisions for organizations that implement recognized best practices such as external validation, bias monitoring, limitation disclosures, and post-deployment monitoring. De-identified, aggregate analysis of AI performance and overrides should be encouraged and/or required for oversight and/or quality improvement initiatives.

5. Strengthen federal investments in AI research, benchmarking, and post-deployment monitoring. To accelerate safe adoption, HHS should prioritize applied research on model drift detection and calibration, shared benchmarking datasets and evaluation methods, and public-private partnerships to study real-world performance, fairness, and usability in clinical workflows.

RFI Question 1: What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

From Veradigm's perspective, the most significant barriers fall into several related categories: regulatory ambiguity, governance expectations that do not reflect the realities of resource constrained providers, misaligned payment incentives, and workflow readiness challenges.

Independent and rural providers are heavily affected by regulatory fragmentation, as they seldom have compliance teams capable of interpreting varying state and federal frameworks. Uncertainty about how existing oversight applies to any use of AI in a practice often results in delayed decisions, even when AI has clear clinical value. A harmonized federal posture that differentiates between device and non-device AI, and that scales requirements to risk and organizational capacity, would substantially reduce hesitation among small providers. The deregulation efforts in the proposed HTI-5 rule that remove source attribute and model card requirements from Certified Health IT create uncertainty for both vendors and providers around what kind of information to expect and how to compare the various AI tools and functionality that might be available. This uncertainty increases risk for developers and clinicians evaluating clinical decision support tools.

In addition, Veradigm has observed that governance frameworks proposed in various policy discussions often assume the presence of cross functional oversight committees or formalized validation processes that small practices simply cannot support. While robust governance is essential, prescriptive structures risk creating barriers for precisely the organizations that could benefit most from burden reducing tools. Rightsizing expectations for deployers based on organizational scale and the risk profile of the AI tool would advance adoption while protecting patient safety. Payment model misalignment presents another barrier. Independent and rural practices operate on thin margins, making it difficult to invest in even high value AI tools without a clear path to recoup the investment. Payment model reform that could meaningfully impact the use of AI is especially critical to incentivize the adoption and use of AI for independent practices.

RFI Question 2: What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care?

Veradigm encourages HHS to establish a formal federal framework tailored specifically to high-risk clinical AI, one that supports innovation while ensuring patient safety and preserving clinical judgment. Such a framework should provide clear expectations for transparency, documentation, risk assessment, and lifecycle monitoring while explicitly scaling requirements to reflect organizational size. This is particularly important for small and independent practices that cannot absorb the cost of large governance programs.

HHS can also accelerate adoption by harmonizing oversight across agencies including ASTP/ONC, FDA, and CMS, reducing administrative complexity and creating predictable expectations for developers and providers alike. This harmonization should include consistent definitions, align documentation requirements, and coordinate expectations for intended use statements, monitoring practices, and safety claims.

Equally critical is payment modernization. CMS should consider add-on payments, AI specific credits within quality programs, and pathways in alternative payment models that reward demonstrated clinical value, burden reduction, or cost savings. Clear reimbursement signals are necessary for small and rural providers who otherwise cannot justify the upfront investment.

RFI Question 3: For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to address them?

The ongoing development of clinical AI that is considered Non-Device CDS by FDA introduces new questions related to liability, contracting, data use under HIPAA, and end user readiness. For independent and rural providers that lack legal and compliance resources, this uncertainty can pose a significant adoption barrier.

A key challenge is the allocation of responsibility among developers, implementers, and clinicians when AI recommendations inform care decisions. Because these tools do not fall under FDA's traditional regulatory structure, EHR vendors like Veradigm look to requirements for Certified Health IT that clarify obligations around clinical decision support tools that are embedded in the EHR. The recent FDA Guidance for Industry and Food and Drug Administration Staff, published on January 29, 2026, related to clinical decision support in non-medical devices provides clear and meaningful examples of functionality that falls in and outside of the FDA purview, and is helpful in terms of understanding what may be outside the scope of FDA oversight, but there remains a gap in terms of the specifics of documentation and information that must be provided to clinicians in order for them to be considered fully informed about a tool and/or how it was developed. We welcome more certainty on the obligations of EHR vendors who are developing or deploying AI, as well as additional guidance that can provide clarity on novel AI workflows and functionality to ensure uniform understanding of the obligations developers have to end users.

HHS could further support responsible adoption by convening stakeholders to develop model liability frameworks and safe-harbor approaches for documenting AI-informed decisions, ensuring accountability aligns with real-world roles. We encourage any stakeholder group to include independent physician practices and ambulatory EHR developers to ensure adequate representation of the specific nuances of AI use in these care settings.

Additionally, there remains ambiguity around permissible uses of deidentified and limited data sets for AI development and monitoring under HIPAA. HHS clarification of allowable pathways, coupled with encouragement of privacy preserving methods such as federated learning, would help providers support AI safety without compromising patient privacy.

Finally, the move toward AI augmented workflows raises questions related to training, credentialing, and scope of practice. Alignment across CMS participation requirements, HRSA workforce programs, and ONC safety expectations would provide much needed clarity for organizations with limited training resources.

Question 4: For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions).

Evaluation of non-device AI tools must include a combination of technical validation and real-world workflow assessment. Before deployment, developers and providers should define intended use, validate models across representative populations and conduct human factors testing in realistic EHR workflows. After deployment, monitoring for drift, calibration, disparities, and user trust should continue, but in a way that small practices can feasibly manage. We advocate for flexible regulations that allow institutions, both developers and deployers, to develop safeguards that align with organizational priorities, capabilities and the commensurate risk of the AI functionality that is being developed and/or used. Overly prescriptive pre- and post-deployment testing runs the risk of stifling innovation and adding unnecessary burden for low-risk AI tools that have the potential to improve administrative burden for provider practices.

HHS can support consistency and feasibility by funding shared evaluation frameworks, national benchmarks, and transparent reporting templates that organizations of all sizes can adopt.

Question 5: How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

Some independent practices, including many Veradigm customers, are already developing internal governance and safety frameworks. However, these efforts are uneven and can be difficult for small providers to navigate and maintain. HHS recognition of voluntary accreditation and certification models, aligned with federal principles, would strengthen these frameworks and create more uniform expectations across the market. Requiring uniform and standardized documentation requirements would also promote consistency, transparency and trust.

Question 7: Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Across the healthcare system, decisions about adopting AI tools typically rest with clinical leadership, IT departments, compliance teams, and quality and safety committees or some combination thereof. However, in independent physician practices, small provider groups, and many rural health systems, these responsibilities are often consolidated into one or two individuals who are already managing significant clinical and administrative workload. This creates a structural barrier to evaluating, procuring, and safely implementing new AI tools.

HHS can play a pivotal role in reducing these hurdles by establishing clear, scalable federal documentation and governance expectations that small practices can implement without requiring dedicated committees or specialized staff. In addition, HHS could encourage or standardize pilot/test environments that allow smaller practices to evaluate AI safely without committing immediately to full deployment, something many decisionmakers view as essential but often lack the infrastructure to support. HHS could also align CMS program requirements and procurement expectations, reducing uncertainty around what evidence or documentation a practice must maintain when adopting AI.

Finally, federal support for simple, EHR embedded guidance, implementation toolkits, and workflow ready templates would materially aid independent practices that rely on vendors for practical, turnkey solutions that fit within existing budget cycles and operational constraints.

Question 8: Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

AI's effectiveness depends heavily on consistent, high-quality data, yet many practices lack access to the robust interoperability infrastructure that large systems use. HHS can address this by continuing to advance consistent, enforceable API requirements and continuing to advance data elements supported by USCDI ensuring that AI developers and clinicians can rely on predictable and complete data sources.

To further support responsible AI adoption, HHS should accelerate expansion of data elements represented in USCDI and specialty or domain specific data elements such as those in USCDI+ that reflect real clinical needs for AI use cases. Additionally, HHS can drive equity and transparency by funding and maintaining national benchmarking criteria and evaluation datasets and enabling providers of all sizes to compare model performance with consistent criteria. These steps would significantly strengthen model reliability, external validation, and fairness while reducing the burden on small practices that cannot independently generate or curate large-scale evaluation datasets or domain-specific data elements that reflect real clinical needs.

Question 9: What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

For clinicians, particularly those in independent practices, meeting patient expectations for transparency and privacy expectations when AI is used at the point of care requires guidance, educational resources, and clarity about communication obligations.

HHS can support these needs by developing plain language, patient-facing educational materials explaining when and how AI may be used, what protections should be in place, how privacy is safeguarded, and what recourse or options patients have if they have questions or concerns. Federal leadership could also establish baseline expectations for transparency and clinician communication, giving providers, especially those with limited staff, clear, accessible resources that build trust without adding undue administrative burden. Additionally, HHS can reinforce patient confidence by clarifying how existing privacy protections apply to AI workflows and by supporting common sense safety, and performance monitoring requirements that reassure patients their care remains under appropriate oversight.

Question 10: Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

AI research has historically focused on algorithmic performance in controlled settings, but widespread and equitable adoption requires deeper understanding of how AI performs in the real world, especially in diverse practice settings such as independent clinics and rural communities. HHS can address this

need by funding longitudinal real-world performance and safety studies that assess calibration, drift, disparities, and clinical outcomes across different care environments. This is critical for providers who lack the data science capacity to independently evaluate these issues.

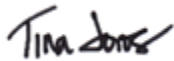
HHS should also invest in economic impact analyses that quantify how AI affects clinician workload, administrative burden, operational efficiency, and cost of care; evidence that is especially important for small practices with limited financial margin. Finally, HHS can advance understanding of human–AI teaming and workflow integration, providing evidence-based guidance on expectations for AI systems that meaningfully support clinicians rather than adding new burdens. These federal research investments would create a stronger foundation for payment policy, regulatory expectations, and industry best practices.

Conclusion

Veradigm appreciates HHS’s leadership in developing a forward-thinking and equitable approach to clinical AI. With harmonized expectations, scalable governance, strengthened transparency, support for privacy preserving data use, and modernized payment pathways, HHS can create an environment where AI improves outcomes, reduces burden, and enhances sustainability across all care settings.

We stand ready to collaborate with HHS to support the safe, effective, and equitable deployment of AI nationwide.

Respectfully,



Tina Joros, JD

Vice President – Policy & Innovation

Veradigm LLC